PRACTICE EXAM 10 / EXAM SET 02

Domain 3: Clinical Patient Care

84 scored questions	4 answer choices per question	Answer key + explanations included

Instructions

Choose the single best answer (A, B, C, or D) for each question. Mark your responses before checking the answer key and detailed explanations at the end of this document. These practice questions are not official NHA examination items.

QUESTIONS

Q01 | General Patient Care

> **SCENARIO** A patient is alert and able to swallow, and a confirmed glucose result is 58 mg/dL. Applicable law and the supervising provider's written standing order authorize this trained assistant to give 15 g oral glucose for values below 70 mg/dL in such patients, notify the clinician, and recheck in 15 minutes.

Which action follows the standing order?

A. Give the authorized 15 g oral glucose, notify the clinician, and recheck in 15 minutes.

B. Offer a sugar-free drink instead of the specified 15 g oral glucose.

C. Give the authorized glucose but omit notification and the scheduled recheck if symptoms improve.

D. Notify the clinician but delay the explicitly authorized oral glucose and reassessment.

Q02 | Infection Control and Safety

> **SCENARIO** A fit-tested assistant dons the assigned respirator for a task requiring it. The manufacturer's required user seal check fails repeatedly despite the permitted adjustments, and the clinic has a process for obtaining an alternate suitable respirator.

What should the assistant do?

A. Enter based on the prior fit test without resolving the failed seal check.

B. Remove the respirator in the exposure area to readjust it there.

C. Use unapproved tape on the leaking edge and proceed with the task.

D. Do not enter the exposure area; obtain a suitable respirator through the approved process.

Q03 | General Patient Care

> **SCENARIO** A patient presents with a visibly deformed wrist after a fall. The provider is on the way, and the assistant is trained to support injured limbs but is not authorized to reduce fractures.

How should the assistant help?

A. Support the wrist in the position found and avoid attempted realignment while obtaining prompt evaluation.
B. Ask the patient to rotate the wrist through its full range of motion.
C. Apply traction until the deformity disappears before notifying the provider.
D. Pull the wrist straight so it fits a standard splint.

Q04 | General Patient Care

> **SCENARIO** A stable patient's scheduled video visit cannot connect because the camera fails. Clinic policy allows the assistant to troubleshoot and contact the provider to determine whether an approved alternative visit method is suitable.

What should the assistant do?

A. Tell the patient to obtain treatment advice from the assistant instead of the provider.
B. Document a completed video examination despite the failed connection.
C. Attempt the approved connection steps and contact the provider about an appropriate alternative.
D. Promise that an audio call always provides the same examination as video.

Q05 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A swabbed specimen arrives with a label identifying the patient, but the requisition names two different tests and the signed order authorizes only one. The laboratory requires clarification before processing an ambiguous request.

What should the assistant do?

A. Send the specimen with the discrepancy unresolved and ask the laboratory to choose.
B. Request both listed tests to avoid a second collection without obtaining authorization.
C. Choose the test that best matches the patient's complaint without clinician clarification.
D. Resolve the requested test against the signed order and obtain any needed authorized clarification.

Q06 | General Patient Care

> **SCENARIO** A provider plans to use electrosurgery for a small skin procedure. The preparation product instructions require complete drying and removal of pooled solution before an ignition source is used. The assistant sees wet prep solution in a skin fold.

What should the assistant do?

A. Wipe only the visible outer skin and assume the pooled solution has evaporated.
B. Continue setup and mention the pooling after electrosurgery has begun.
C. Cover the wet fold with a dry drape and tell the provider the site is ready.
D. Alert the provider and ensure the supplied drying and pooling precautions are completed before use.

Q07 | Point of Care Testing and Laboratory Procedures

SCENARIO During provider-directed skin allergy testing, the measurement instruction is to record the raised wheal in millimeters without substituting the surrounding redness. The wheal measures 5 mm and the surrounding red area measures 14 mm.

Which finding belongs in the wheal-size field?

A. 19 mm
B. 14 mm
C. 9 mm
D. 5 mm

Q08 | General Patient Care

SCENARIO A trained assistant is authorized to give an ordered oral liquid dose of 12 mg/kg once to a child whose verified weight is 24 kg. The verified medication contains 40 mg/mL. The order is complete and has been clinically verified; round only the final volume to one decimal place.

What volume provides the ordered dose?

A. 0.3 mL
B. 7.2 mL
C. 0.6 mL
D. 72 mL

Q09 | Infection Control and Safety

SCENARIO A reusable instrument has a cracked joint that traps visible material. The manufacturer's inspection instructions require removing instruments with this defect from service; a replacement is available.

What should the assistant do?

A. Use the instrument if its outer surfaces look shiny.
B. Run a longer sterilization cycle to compensate for the cracked joint.
C. Cover the crack with tape and package the instrument.
D. Remove the damaged instrument from service and obtain the suitable replacement.

Q10 | Patient Intake and Vitals

> **SCENARIO** An adult weighs 81 kg and is 1.8 m tall. The intake tool defines BMI = weight in kg / height in m squared and asks for one decimal place.

What BMI should the assistant enter?

A. 26.2 kg/m2
B. 20.3 kg/m2
C. 45.0 kg/m2
D. 25.0 kg/m2

Q11 | General Patient Care

> **SCENARIO** A patient develops a nosebleed while awaiting examination. The clinic first-aid protocol for an alert, stable patient specifies sitting upright, leaning forward, and compressing the soft part of the nose while notifying clinical staff.

Which position should the assistant help the patient assume?

A. Prone with the face pressed into a pillow.
B. Upright with the head leaning forward during the instructed compression.
C. Supine with the head tilted backward.
D. Standing with the head extended upward.

Q12 | General Patient Care

> **SCENARIO** An assistant trained and authorized for ordered ear irrigation checks the solution temperature. The clinic procedure specifies 36-38 degrees C for this solution; the prepared bottle measures 21 degrees C.

What should happen before use?

A. Use the 21-degree solution because any clean solution is acceptable.
B. Prepare solution within the specified temperature range and recheck it.
C. Heat the solution above 45 degrees C to compensate for the initial temperature.
D. Increase irrigation pressure to make the cool solution more effective.

Q13 | Infection Control and Safety

> **SCENARIO** A cleaned hinged surgical instrument is being packaged for sterilization. Its validated instructions require the hinge to remain open and unlocked throughout processing.

How should it be packaged?

A. With the hinge open and unlocked as specified.
B. With the jaws wrapped shut in adhesive tape.
C. With the ratchet locked tightly to minimize movement.
D. With the hinge closed and covered by another instrument.

Q14 | Infection Control and Safety

> **SCENARIO** The outside of a used glove is contaminated with blood. During removal, the assistant wants to avoid transferring that contamination to the bare opposite hand.

Which removal action is appropriate?

A. Remove the gloves and handle clean equipment before hand hygiene.
B. Pinch the bloody glove exterior with bare fingertips.
C. Keep bare fingers away from the contaminated outer glove surface during removal.
D. Use the contaminated glove exterior to wipe the opposite wrist.

Q15 | Phlebotomy

> **SCENARIO** A blood-culture collection protocol requires repeat antisepsis if the prepared site is touched by a nonsterile glove before puncture. After the full initial preparation, the assistant palpates the site with a nonsterile glove.

What should happen before collection?

A. Wipe once with dry gauze and consider antisepsis restored.
B. Proceed because the glove looks clean.
C. Touch the site again to confirm the vein and puncture immediately.
D. Repeat the required antisepsis and drying under the protocol.

Q16 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A patient receives a specified fecal immunochemical test kit. Its supplied instructions state no dietary restriction, but the patient remembers avoiding certain foods for a different stool test years ago.

What should the assistant reinforce?

A. Follow the current kit's instructions and ask the clinician about medication questions.
B. Stop prescribed medication because the old test required preparation.
C. Apply the old dietary restrictions to every stool blood test.
D. Discard the kit because two stool tests cannot have different instructions.

Q17 | Infection Control and Safety

A reusable mucous-membrane-contact device has validated instructions requiring cleaning followed by high-level disinfection. Which processing method follows those instructions?

A. Detergent cleaning alone followed by storage.
B. Cleaning followed by the validated high-level disinfection process.
C. A brief low-level surface wipe without prior cleaning.
D. Any household cleaner chosen by scent.

Q18 | EKG and Cardiovascular Testing

At an EKG gain of 10 mm/mV, defined as 10 vertical millimeters per 1 mV, what voltage does a 5 mm vertical deflection represent?

A. 50 mV
B. 0.5 mV
C. 2 mV
D. 5 mV

Q19 | General Patient Care

> **SCENARIO** A provider orders a thin layer of topical cream to a specified intact-skin area. The assistant is trained and authorized to apply it, and the product instructions require gloves and no occlusive covering.

Which application matches the order and instructions?

A. Spread the cream beyond the specified area to use the whole container.
B. Use gloves to apply a thin layer to the specified area and leave it uncovered.
C. Apply a thick layer and seal it under plastic wrap.
D. Apply the cream with bare fingers and cover it with an airtight dressing.

Q20 | Patient Intake and Vitals

> **SCENARIO** A seated adult has a measured respiratory rate of 26 breaths/min. The supplied rooming protocol flags rates above 24 breaths/min for prompt clinician notification. The patient is speaking comfortably and has no new distress.

What should the assistant do?

A. Omit the rate because no emergency symptoms are present.
B. Replace the rate with 18 because the patient can speak comfortably.
C. Diagnose a respiratory infection from the rate alone.
D. Document the measured rate and promptly notify the clinician under the protocol.

Q21 | Phlebotomy

> **SCENARIO** During a capillary finger collection, very little blood flows. The test instructions warn against forceful repeated squeezing and permit warming and recollection with a new lancet when needed.

Which approach best protects specimen quality?

A. Add water to the small drop so it reaches the required volume.
B. Avoid forceful milking and use the permitted warming and recollection steps.
C. Squeeze the finger as hard as possible until enough fluid is obtained.
D. Scrape the skin with the collection tube to increase flow.

Q22 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** An ordered sputum test requires material produced by a deep cough from the lower respiratory tract. The supplied collection instructions distinguish sputum from saliva. The patient offers to spit into the cup without coughing.

Which instruction should the assistant reinforce?

A. Use the instructed deep-cough technique to collect sputum rather than saliva.
B. Add drinking water to saliva to make the specimen less thick.
C. Collect nasal drainage on a tissue and place the tissue in the cup.
D. Fill the cup with saliva because all mouth secretions are equivalent.

Q23 | Infection Control and Safety

> **SCENARIO** A patient is under the clinic's specified contact precautions. The posted procedure calls for patient-dedicated equipment whenever available, and a dedicated blood pressure cuff is in the room.

Which equipment choice follows the procedure?

A. Use the dedicated cuff and keep it within the designated patient area.
B. Carry the dedicated cuff directly to another patient without processing.
C. Return the used dedicated cuff to clean storage before discharge cleaning.
D. Replace it with a shared cuff so several rooms use the same supplies.

Q24 | Patient Intake and Vitals

> **SCENARIO** Before an ordered manual blood pressure measurement, the assistant palpates a systolic estimate of 148 mm Hg. The supplied procedure directs inflation to 30 mm Hg above that estimate before auscultation.

To which pressure should the cuff be inflated under this procedure?

A. 208 mm Hg
B. 158 mm Hg
C. 148 mm Hg
D. 178 mm Hg

Q25 | Infection Control and Safety

> **SCENARIO** After removing gloves used for a contaminated task, the assistant sees visible soil on the hands. Soap, running water, and alcohol-based hand rub are available.

Which hand-hygiene method is appropriate?

A. Wash with soap and running water.
B. Put on two pairs of gloves to cover the soil.
C. Apply hand rub over the visible soil without washing.
D. Wipe the hands on a clean gown and proceed.

Q26 | Infection Control and Safety

> **SCENARIO** A used cloth examination cover is wet with blood. Clinic laundry procedure requires minimal handling and a designated leak-resistant labeled laundry bag at the point of use.

How should the assistant handle the cover?

A. Rinse it by hand in a clean medication-preparation sink.
B. Place it in a clean linen cupboard until it dries.
C. Shake it vigorously before carrying it through the corridor.
D. Bag it with minimal handling in the designated laundry container.

Q27 | General Patient Care

> **SCENARIO** An assistant trained and authorized for intradermal testing administers a test dose. The clinic procedure requires a visible wheal and directs staff to notify the clinician if a wheal does not form. No wheal appears.

What should the assistant do?

A. Document the observation and obtain clinician instructions under the procedure.
B. Assume the dose was correct and record that a wheal was present.
C. Independently inject an additional full dose at the same site.
D. Rub the site vigorously until it becomes swollen.

Q28 | Phlebotomy

> **SCENARIO** The assistant collected venous blood with a syringe using an authorized method. The laboratory supplies an approved needleless tube-transfer device and directs staff not to push the syringe plunger forcibly during transfer.

Which method follows the instruction?

A. Force the plunger rapidly to fill the evacuated tube faster.
B. Remove the tube stopper and pour blood into it from above.
C. Use the approved transfer device and allow transfer as directed without forceful plunger pressure.
D. Puncture each tube stopper with an exposed collection needle held by hand.

Q29 | General Patient Care

> **SCENARIO** A provider has directed preparation of an electronic prescription for review, but the draft remains unsigned. Clinic workflow requires the prescriber's electronic authorization before transmission.

What should the assistant do?

A. Route the completed draft to the prescriber for authorization.
B. Transmit it using the prescriber's login without the prescriber's review.
C. Mark the draft signed because its fields are complete.
D. Send a changed dose to the pharmacy to make the draft actionable.

Q30 | Patient Intake and Vitals

> **SCENARIO** An older adult with advanced dementia cannot reliably use a numeric pain scale. During movement, the patient grimaces and guards the hip. Clinic procedure specifies a validated observational pain tool when self-report is not reliable.

Which approach follows the procedure?

A. Record zero pain because no numeric score can be obtained.
B. Record the patient as refusing care and omit the behaviors.
C. Assign a numeric score based solely on the caregiver's facial expression.
D. Use the specified observational tool and document the observed behaviors.

Q31 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A patient collecting a timed 24-hour urine specimen reports missing one void. The laboratory instructions state that incomplete collections must be discussed with the laboratory before submission and must not be labeled complete.

What should the assistant do?

A. Submit the collection as complete but tell the patient to mention the missed void later.
B. Estimate the missed volume and add water before labeling the collection complete.
C. Extend the collection by one additional void without obtaining laboratory instructions.
D. Report the missed void to the laboratory and obtain instructions without labeling the collection complete.

Q32 | Patient Intake and Vitals

> **SCENARIO** The intake form asks about tobacco and nicotine use. A patient says cigarettes were stopped six months ago but a nicotine vape is used every day.

How should the assistant record this history?

A. Mark no nicotine use because cigarettes were stopped.
B. Delete the tobacco section because vaping is not a prescription medication.
C. Document former cigarette use and current reported nicotine vaping.
D. Record current cigarette smoking without mentioning the vape.

Q33 | Infection Control and Safety

> **SCENARIO** After a sterilizer cycle, external chemical indicator tape has changed color. A coworker says the tape alone proves that every item in every package is sterile.

Which interpretation should the assistant use?

A. The tape directly detects every surviving organism inside the instruments.

B. Any color change overrides an incomplete cycle record.

C. The tape indicates exposure to specified conditions but does not alone prove sterility of every item.

D. Changed external tape makes internal indicators and cycle monitoring unnecessary.

Q34 | Infection Control and Safety

> **SCENARIO** A blood-contaminated glass tube breaks on the floor. The trained assistant has a dustpan, brush, and approved sharps container, and clinic procedure prohibits picking up contaminated broken glass by hand.

Which cleanup method follows the procedure?

A. Pick up the fragments with gloved fingers because gloves prevent cuts.

B. Wipe the fragments into a regular wastebasket with a bare hand.

C. Use the brush and dustpan to collect fragments into the approved sharps container.

D. Leave fragments in a laundry bag for the laundry staff to sort.

Q35 | Phlebotomy

> **SCENARIO** A 4 kg infant has several requested blood tests. The provider's written collection limit for this visit is 0.5 mL/kg total, and the listed tube requirements sum to 3 mL. The assistant may not change the orders or exceed the limit.

What should the assistant do before collection?

A. Collect all 3 mL in separate draws because each draw is below the 2 mL limit.

B. Use standard adult tubes and accept whatever total volume they draw.

C. Select which ordered test to omit without contacting the clinician.

D. Report that the 2 mL limit is below the 3 mL requirement and obtain a revised authorized collection plan.

Q36 | General Patient Care

> **SCENARIO** An assistant is authorized to administer a prescribed oral medication. After receiving an explanation, the alert patient declines the dose. No medication has been administered.

What should the assistant do?

A. Hide the dose in food so the patient receives the ordered treatment.

B. Document the refusal and notify the clinician without recording the dose as given.

C. Choose a different drug and offer it without a new order.

D. Record the dose as administered because it was prepared.

Q37 | EKG and Cardiovascular Testing

SCENARIO An EKG electrode repeatedly lifts from a densely haired area. The device instructions allow clipping excess hair as needed, then cleaning and drying the skin before applying a fresh electrode.

Which preparation follows the instructions?

A. Place several loose electrodes on top of one another without skin preparation.
B. Apply the electrode over body lotion to make it slide into position.
C. Clip as needed and prepare clean, dry skin before applying a fresh electrode.
D. Scrape the skin until it bleeds so the electrode has a firmer surface.

Q38 | Point of Care Testing and Laboratory Procedures

SCENARIO Before hearing screening begins, the assistant notices that the transducer labeled right is on the left ear and the transducer labeled left is on the right. The device instructions require matching ear labels.

What should the assistant do?

A. Assume reversing the headset cannot affect ear-specific documentation.
B. Correct the earphone placement before beginning the screening.
C. Leave them reversed and report all results under the printed ear labels.
D. Test with one earpiece between the two ears.

Q39 | Phlebotomy

SCENARIO During a provider-ordered timed glucose tolerance procedure, the patient vomits before finishing the required drink. The test instructions direct staff to stop the timed sequence and contact the ordering clinician if the full drink is not retained.

What should the assistant do?

A. Record the full drink as retained and collect all scheduled samples.
B. Estimate a final glucose result from the amount the patient drank.
C. Prepare another full drink and restart independently.
D. Record what occurred, stop the sequence, and obtain clinician instructions.

Q40 | Patient Intake and Vitals

SCENARIO A thermometer displays 38.5 degrees C. The EHR field requires degrees F and supplies the formula F = (C x 9/5) + 32, rounded to one decimal place.

Which value should be entered?

A. 53.4 degrees F
B. 107.0 degrees F
C. 69.3 degrees F
D. 101.3 degrees F

Q41 | Phlebotomy

> **SCENARIO** A request arrives for an arterial blood specimen. This clinic assistant is trained and authorized only for venous and capillary collection; a qualified arterial collector is available through the clinic.

What should the assistant do?

A. Refer the requested collection to the qualified arterial collector through the clinic workflow.
B. Change the specimen requirement to capillary without authorization.
C. Collect venous blood and label it arterial to avoid delay.
D. Attempt arterial puncture because a provider requested the test.

Q42 | General Patient Care

> **SCENARIO** During medication verification, a barcode alert identifies a different drug from the signed order. The two product names look similar, and the patient has not received either product.

What should the assistant do?

A. Set the alerted product aside but give a similar-looking package without repeating verification.
B. Stop preparation and reconcile the product identity with the order using the approved workflow.
C. Proceed after a coworker confirms the packaging color without matching the full order.
D. Override the alert after matching only the first several letters of the product name.

Q43 | Patient Intake and Vitals

> **SCENARIO** At rooming, a patient says, "My stomach has burned after meals for three days." No diagnosis has been made. The chief-complaint field is blank.

Which entry best records the reason for the visit?

A. New diagnosis of a peptic ulcer.
B. Gastrointestinal examination normal.
C. Reports stomach burning after meals for three days.
D. Follow-up of a confirmed medication adverse effect.

Q44 | General Patient Care

> **SCENARIO** A patient arrives with a small metal fragment visibly embedded in the eye after grinding metal. The assistant is not authorized to remove embedded ocular objects, and the clinician is immediately available.

Which action is appropriate?

A. Attempt to lift the visible fragment with a damp swab before notifying the clinician.
B. Place a firm pressure dressing on the injured eye while awaiting evaluation.
C. Protect the eye from pressure and summon the clinician immediately.
D. Begin routine irrigation before reporting that the fragment appears embedded.

Q45 | General Patient Care

> **SCENARIO** The assistant is preparing an authorized injection. The selected prefilled product is past its printed expiration date; an in-date supply of the ordered product is available.

Which action is appropriate?

A. Use the expired product if the solution is clear.
B. Remove the expired product from use and verify an in-date replacement.
C. Change the printed expiration date to today's date.
D. Give an increased volume to compensate for possible potency loss.

Q46 | Patient Intake and Vitals

> **SCENARIO** An empty mechanical clinic scale reads 3 lb before a patient steps on it. The scale instructions require checking and adjusting zero before use.

What should the assistant do before weighing the patient?

A. Set and verify zero according to the scale instructions.
B. Record the previous visit's weight until the scale is replaced.
C. Weigh the patient and add 3 lb to the displayed value.
D. Ignore the offset if the patient has used this scale before.

Q47 | Infection Control and Safety

> **SCENARIO** An evacuation alarm sounds in an ambulatory clinic. The clinic plan requires the nearest safe marked exit; one nearby exit route contains smoke, while a second marked ground-floor route is clear. Trained staff are available to assist patients.

Which evacuation action follows the plan?

A. Block the clear exit with supply carts before starting evacuation.
B. Keep all patients in the waiting room until every chart is printed.
C. Direct and assist patients through the clear marked route under the evacuation plan.
D. Use the smoky route because it is a few steps shorter.

Q48 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** For an ordered peak-flow assessment, the provider-approved protocol requires three technically acceptable attempts and recording the highest value. The patient produces acceptable values of 350, 380, and 360 L/min.

Which value should the assistant record as the protocol result?

A. 360 L/min
B. 350 L/min
C. 380 L/min
D. 363.3 L/min

Q49 | General Patient Care

SCENARIO During a telehealth intake, a patient reports a home blood pressure of 132/78 mm Hg measured that morning. The assistant did not observe the measurement. The EHR has separate fields for patient-reported and clinic-measured data.

Where should the value be recorded?

A. In the clinic-measured field as a directly observed office measurement.
B. As an average with the previous office measurement without retaining the source.
C. In the patient-reported field with the reported time and home source.
D. As an unqualified provider-confirmed measurement.

Q50 | Infection Control and Safety

SCENARIO The sharps container beside the injection station has reached its marked fill line. Clinic procedure requires replacement at that line, and an unused approved container is available.

What should the assistant do before the next injection?

A. Leave the next used needle on a tray until the container is emptied.
B. Replace the filled container using the approved closure and handling procedure.
C. Transfer loose sharps into a regular waste bag to reuse the container.
D. Push the contents down to create space below the line.

Q51 | General Patient Care

SCENARIO A provider authorizes a subcutaneous dose using a specified prefilled device. Its instructions require insertion at 90 degrees into a skin fold and prohibit massaging afterward. The trained assistant has confirmed the appropriate site.

Which technique follows these device instructions?

A. Insert through clothing to maintain the selected site.
B. Empty the device into a different syringe before verifying its markings.
C. Insert at 90 degrees into the skin fold and avoid massaging afterward.
D. Insert at 15 degrees and rub the site firmly afterward.

Q52 | General Patient Care

SCENARIO An authorized injection order specifies 0.26 mL. Available syringes have readable graduations of 0.01 mL, 0.1 mL, 0.5 mL, and 1 mL. The assistant must measure the ordered volume without rounding.

Which graduation allows the required measurement?

A. 1 mL graduations.
B. 0.1 mL graduations.
C. 0.5 mL graduations.
D. 0.01 mL graduations.

Q53 | Patient Intake and Vitals

For infant head circumference, the supplied technique uses a nonstretch tape around the greatest occipitofrontal circumference, above the eyebrows and ears and over the most prominent occiput. Which tape placement follows that technique?

A. Around the neck just below the jawline.
B. Around the widest circumference at the stated frontal and occipital landmarks.
C. Around the forehead alone without including the occiput.
D. Over the crown and under the chin.

Q54 | Infection Control and Safety

> **SCENARIO** The clinic's posted PPE donning procedure for a specified task is gown, mask with eye protection, then gloves extending over the gown cuffs. The assistant has already donned the gown and mask with eye protection.

What should be done next?

A. Put gloves under the sleeves with the wrists exposed.
B. Skip gloves because the gown covers the forearms.
C. Remove the gown before putting on gloves.
D. Put on gloves so they cover the gown cuffs.

Q55 | Patient Intake and Vitals

> **SCENARIO** A patient uses a walker and screens positive for fall risk. The examination room has a low adjustable chair, a reachable call device, and a loose electrical cord across the walking path. The patient needs to wait for the provider.

Which rooming action best addresses the immediate fall hazard?

A. Clear the cord and leave the patient safely seated with the walker and call device accessible.
B. Leave the patient standing beside the table to avoid a later transfer.
C. Place the walker outside the closed door to create more floor space.
D. Ask the patient to step over the cord whenever assistance is needed.

Q56 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A clinic holds only a CLIA Certificate of Waiver. The requested test system is explicitly classified as moderate complexity, and the clinic does not have authority to perform it under another certificate.

What should the assistant do with the request?

A. Run the test but label its result as a waived test.
B. Assume the certificate expands when a provider signs the order.
C. Route the specimen or request to an appropriately authorized laboratory through the approved process.
D. Perform the test in the clinic because its instrument is small.

Q57 | Infection Control and Safety

> **SCENARIO** An approved disinfectant concentrate must be mixed as one part concentrate plus nine parts water. The product instructions specify 10 mL concentrate for a 100 mL final batch.

Which mixture matches the instructions?

A. 10 mL concentrate plus 100 mL water.
B. 1 mL concentrate plus 99 mL water.
C. 10 mL concentrate plus 90 mL water.
D. 90 mL concentrate plus 10 mL water.

Q58 | Patient Intake and Vitals

> **SCENARIO** A clinic orders orthostatic measurements with blood pressure and pulse documented after five minutes supine and at one and three minutes standing. A stable patient completes the procedure. The assistant has all values on a worksheet.

Which EHR entry best preserves the measurement context?

A. Record each value with its body position and the specified measurement time.
B. Label all measurements as seated because the visit began in a chair.
C. Record only the highest pressure without the body position.
D. Average all supine and standing values into one unlabeled pressure.

Q59 | EKG and Cardiovascular Testing

> **SCENARIO** At home with an ambulatory monitor, a patient develops severe chest pressure and shortness of breath. The provider's written instructions say to call 911 for these symptoms and state that the recorder is not continuously watched in real time. The patient phones the assistant and plans to wait for the monitor team to call.

Which response follows the instructions?

A. Tell the patient to call 911 now and follow the clinic's emergency-call procedure.
B. Ask the patient to wait for a monitor-team callback before contacting emergency services.
C. Record the event and offer a routine clinic appointment later in the day.
D. Tell the patient that pressing the recorder button replaces an emergency call.

Q60 | General Patient Care

> **SCENARIO** For a dose-verification exercise before authorized oral administration, the provider's order limits total acetaminophen from all products to 3,000 mg in the defined 24-hour period. The medication record shows 1,950 mg already given, and the currently ordered dose is 650 mg.

What total would result after the currently ordered dose?

A. 2,600 mg
B. 3,900 mg
C. 1,300 mg
D. 3,250 mg

Q61 | Phlebotomy

> **SCENARIO** An EDTA blood specimen contains visible clots. The laboratory acceptance instructions reject clotted specimens for the ordered test, and the clinician needs a valid result.

What should the assistant do?

A. Mix more vigorously until the clots break up, then submit as acceptable.
B. Remove visible clots and submit the remaining blood for the same test.
C. Notify the appropriate staff and arrange a properly collected replacement.
D. Submit it without noting clots and let the laboratory discover the issue.

Q62 | Patient Intake and Vitals

> **SCENARIO** A 4-year-old is roomed for a routine visit. The child is calm, and the clinic protocol calls for an apical pulse count over 60 seconds in this situation. The assistant counts 102 beats in that interval.

Which entry accurately records the measurement?

A. Apical pulse 102 beats/min, counted for 60 seconds.
B. Apical pulse 51 beats/min, divided by two for age.
C. Apical pulse 408 beats/min, calculated from the count.
D. Radial pulse 102 beats/min, counted for 15 seconds.

Q63 | General Patient Care

> **SCENARIO** A provider orders removal of eight skin staples after evaluating a healed incision. A trained, authorized assistant removes seven, then cannot see the eighth. Clinic procedure directs staff to stop and report a count discrepancy rather than probe tissue.

What should the assistant do?

A. Inspect only the removed staples, then discharge without resolving the wound count.
B. Report the discrepancy and have the provider evaluate before proceeding.
C. Probe beneath the skin with the remover to locate the missing staple independently.
D. Chart eight removed based on the referral count and ask the provider to review later.

Q64 | EKG and Cardiovascular Testing

> **SCENARIO** A patient has a right below-knee amputation. The clinic's approved EKG diagram permits leg electrodes on comparable lower-thigh sites when lower-leg sites are unavailable and requires documenting the modification. The assistant is following that diagram.

Which placement and documentation are appropriate?

A. Use comparable lower-thigh sites for both leg electrodes and document the modification.
B. Place both leg electrodes on the same left ankle without noting the change.
C. Leave the right-leg electrode disconnected and document standard placement.
D. Place the right-leg electrode on the chest and assume it is standard.

Q65 | Point of Care Testing and Laboratory Procedures

> **SCENARIO** A specified CLIA-waived test system has failed required QC, and patient testing is already on hold. During troubleshooting, the assistant finds that the control solution expired yesterday. Unexpired compatible control is available; the instructions require acceptable QC with valid materials before testing resumes.

Which troubleshooting action is appropriate?

A. Change the printed control acceptance range to include the failed result.
B. Use the unexpired compatible control and repeat QC as instructed before considering release.
C. Use patient results to decide whether the expired control should be considered acceptable.
D. Repeat QC with the expired control until a passing result appears.

Q66 | General Patient Care

> **SCENARIO** A trained, authorized assistant is administering ordered eye drops. Before a drop is delivered, the bottle tip touches the patient's eyelashes. The clinic policy requires replacement of a contaminated applicator.

Which action follows the policy?

A. Stop and obtain an uncontaminated replacement under the medication workflow.
B. Share a replacement bottle labeled for another patient without verification.
C. Continue because the bottle touched eyelashes rather than the eye itself.
D. Wipe the tip with a uniform sleeve and resume administration.

Q67 | General Patient Care

> **SCENARIO** While pouring sterile solution into a sterile basin, the outside of the bottle touches the inside of the basin. The procedure has not started, and replacement supplies are available.

What should the assistant do?

A. Remove a spoonful of solution where the bottle touched and continue.
B. Continue because the liquid inside the bottle was sterile.
C. Wipe the basin interior with gauze and retain the solution.
D. Replace the contaminated basin and solution before continuing setup.

Q68 | General Patient Care

> **SCENARIO** A provider requests prone positioning for examination of a lesion on the posterior lower leg. The patient can turn safely and has no stated positioning restriction.

How should the assistant prepare the patient?

A. Help the patient lie on the abdomen and drape areas not needed for the examination.
B. Seat the patient with both feet flat and cover the posterior leg.
C. Use lithotomy positioning because the lesion is below the waist.
D. Place the patient supine and expose the entire front of the body.

Q69 | General Patient Care

> **SCENARIO** After a procedure, the provider's written instructions say to keep the dressing dry for 24 hours. The patient plans to shower that evening and asks whether to remove the dressing. No alternative instruction is provided.

What should the assistant do?

A. Reinforce the dry-dressing instruction and obtain clinician guidance about the planned shower.
B. Advise removal and soaking because most dressings are disposable.
C. Independently shorten the dry period to two hours.
D. Tell the patient to replace the instruction with advice from an unrelated prior procedure.

Q70 | General Patient Care

SCENARIO After an assistant applies an ordered elastic forearm wrap, the patient reports new finger tingling and the fingers appear pale. Clinic procedure authorizes the trained assistant to loosen the wrap immediately and notify the clinician when these findings occur.

What should the assistant do?

A. Document the symptoms but leave the wrap unchanged until the scheduled end-of-visit check.
B. Loosen the wrap under the procedure and promptly report the symptoms and appearance.
C. Raise the arm without loosening the wrap or notifying the clinician.
D. Apply another layer over the wrap to prevent slippage before reassessing.

Q71 | General Patient Care

SCENARIO A patient reports a latex allergy before a minor procedure. The tray contains latex examination gloves; approved nonlatex gloves and compatible nonlatex supplies are available.

Which preparation is appropriate?

A. Use latex gloves only during the shortest part of the procedure.
B. Proceed without mentioning the allergy because the assistant will wear the gloves.
C. Wear two pairs of latex gloves to prevent patient contact.
D. Replace latex-containing supplies with approved nonlatex alternatives and alert the procedural team.

Q72 | Phlebotomy

SCENARIO The assistant cleans a routine venipuncture site with the instructed alcohol preparation. The area is still visibly wet, and the product instructions require complete air drying before puncture.

What should the assistant do?

A. Wipe it dry with the patient's sleeve.
B. Puncture through the wet alcohol because drying is optional.
C. Allow the site to air dry fully without touching or blowing on it.
D. Blow across the site to dry it faster.

Q73 | Infection Control and Safety

SCENARIO A reprocessed instrument pack must be traceable to its sterilizer load under clinic procedure. The pack label lacks a load identifier, but several loads were processed that morning.

What should the assistant do before releasing the pack?

A. Copy the number from a similarly wrapped pack without verifying that they shared a load.
B. Resolve its traceability through the approved records and withhold release while identity is unresolved.
C. Assign the last recorded load number because it was processed on the same morning.
D. Release the pack and investigate its load identity after the scheduled procedure.

Q74 | General Patient Care

SCENARIO A patient arrives requesting removal of sutures placed elsewhere. No removal order is present, and clinic policy permits the assistant to remove sutures only after provider evaluation and an order.

What should the assistant do?

A. Arrange provider evaluation and obtain an order before performing removal.
B. Ask reception staff to approve removal in place of a provider order.
C. Use the placement date alone to decide that removal is authorized.
D. Remove the sutures because the patient says the wound is ready.

Q75 | EKG and Cardiovascular Testing

SCENARIO The assistant needs to place V4 during a standard EKG on a patient with breast tissue overlying the required chest landmark. Clinic technique calls for placement on the chest wall at the correct landmark, with privacy and appropriate patient assistance.

Which approach is appropriate?

A. Move every chest electrode to the abdomen to avoid discussing positioning.
B. Use the nipple alone as the landmark without counting intercostal spaces.
C. Explain the need and arrange respectful access to the chest-wall landmark beneath the tissue.
D. Place V4 on top of the breast wherever the electrode adheres most easily.

Q76 | Phlebotomy

SCENARIO A laboratory handling sheet requires a newly collected tube to be transported promptly in an ice-water slurry without freezing. Both a slurry container and a freezer are available.

Which handling choice matches the sheet?

A. Use the specified ice-water slurry and prompt transport without freezing.
B. Place the tube in the freezer until it becomes solid.
C. Transport at room temperature because the distance is short.
D. Store the tube on dry ice instead of following the slurry instruction.

Q77 | Phlebotomy

SCENARIO As a venipuncture needle enters, the patient reports a sudden electric-shock sensation radiating toward the fingers. No tube has filled.

What should the assistant do?

A. Ask the patient to relax while advancing to seek improved blood flow.
B. Stop the attempt, remove the needle safely, and promptly report the radiating pain.
C. Pause the tube collection but leave the needle in place until all supplies are rearranged.
D. Withdraw the needle slightly and continue collection without reporting the radiating pain.

Q78 | General Patient Care

Which instrument is designed to clamp a small bleeding vessel?

A. The scalpel handle.
B. The tissue forceps.
C. The hemostat.
D. The scissors.

Q79 | Phlebotomy

> **SCENARIO** The laboratory's venipuncture procedure limits continuous tourniquet application to 60 seconds and specifies a release-and-rest step before another attempt. The tourniquet has been on for 75 seconds without a puncture.

What should the assistant do?

A. Document 45 seconds so the attempt appears within the limit.
B. Release the tourniquet and follow the specified rest step before another attempt.
C. Tighten it further to shorten the eventual collection.
D. Leave it on until a vein becomes visible regardless of elapsed time.

Q80 | EKG and Cardiovascular Testing

> **SCENARIO** A stable, relaxed patient has a resting EKG with uniform rapid electrical interference. Electrode contact is secure. The interference appears when a nearby nonessential electrical device is powered on and disappears when it is off.

Which action best addresses the observed cause?

A. Ask the relaxed patient to tense the arm muscles during recording.
B. Turn off or relocate the nonessential interference source as permitted by equipment policy and repeat the tracing.
C. Keep the interference and label it as a confirmed cardiac rhythm abnormality.
D. Move all chest electrodes to different intercostal spaces.

Q81 | Patient Intake and Vitals

> **SCENARIO** A patient has a left-arm dialysis fistula and an order specifying use of the right arm for blood pressure. The right arm has no identified restriction, and an appropriately sized cuff is available.

Which measurement site matches the order?

A. The left forearm over the fistula dressing.
B. The left upper arm above the fistula.
C. The right upper arm.
D. Either arm, selected according to the previous chart entry.

Q82 | Phlebotomy

SCENARIO During vein selection, a patient repeatedly pumps the fist vigorously. The laboratory collection instructions allow a gently closed fist but prohibit repeated pumping because it can affect specimen quality.

Which instruction should the assistant give?

A. Keep the fist gently closed without repeated pumping.
B. Alternate hard clenching and release for every tube.
C. Squeeze a resistance ball continuously throughout the draw.
D. Pump faster until blood enters the tube.

Q83 | Patient Intake and Vitals

SCENARIO An automated vital-signs unit displays a low-battery shutdown symbol and turns off before a blood pressure result appears. The patient is comfortable; a working manual device is available, and the assistant is trained in its use.

Which action is appropriate?

A. Record the shutdown symbol as a zero systolic pressure.
B. Use the working manual device and address the powered-off unit under equipment policy.
C. Use the prior patient's last displayed pressure for this visit.
D. Keep restarting the unit and document the first incomplete display as a result.

Q84 | General Patient Care

SCENARIO During ordered suture removal, the trained, authorized assistant follows a procedure that specifies cutting one strand close to the skin before withdrawal to minimize passage of exposed suture through the tissue.

Which technique follows this instruction?

A. Cut both visible strands and leave the buried portion in place without reporting it.
B. Pull firmly on the uncut loop until it breaks beneath the skin.
C. Cut at the top of the knot and draw the entire exposed loop through tissue.
D. Cut close to the skin on one side and withdraw the suture without pulling the long exposed segment through tissue.

ANSWER KEY & EXPLANATIONS

Practice Exam 10 | Domain 3: Clinical Patient Care | 84 scored questions

01 A	02 D	03 A	04 C	05 D	06 D
07 D	08 B	09 D	10 D	11 B	12 B
13 A	14 C	15 D	16 A	17 B	18 B
19 B	20 D	21 B	22 A	23 A	24 D
25 A	26 D	27 A	28 C	29 A	30 D
31 D	32 C	33 C	34 C	35 D	36 B
37 C	38 B	39 D	40 D	41 A	42 B
43 C	44 C	45 B	46 A	47 C	48 C
49 C	50 B	51 C	52 D	53 B	54 D
55 A	56 C	57 C	58 A	59 A	60 A
61 C	62 A	63 B	64 A	65 B	66 A
67 D	68 A	69 A	70 B	71 D	72 C
73 B	74 A	75 C	76 A	77 B	78 C
79 B	80 B	81 C	82 A	83 B	84 D

DETAILED EXPLANATIONS

Q01 | Correct answer: A

Give the authorized 15 g oral glucose, notify the clinician, and recheck in 15 minutes.

The result and ability to swallow meet the explicit standing-order conditions for the specified oral glucose. Notification and the 15-minute recheck remain required parts of the response, including when symptoms improve.

Q02 | Correct answer: D

Do not enter the exposure area; obtain a suitable respirator through the approved process.

The failed required seal check indicates the respirator is not ready for the task. A previous fit test does not eliminate the device-specific check at use or authorize improvised modifications.

Q03 | Correct answer: A

Support the wrist in the position found and avoid attempted realignment while obtaining prompt evaluation.

Supporting the injury without attempting reduction stays within the stated role and limits further injury. Forced movement or traction is not an authorized way to prepare this patient for evaluation.

Q04 | Correct answer: C

Attempt the approved connection steps and contact the provider about an appropriate alternative.

Connection troubleshooting and provider review follow the supplied workflow. The assistant should not falsely document an examination or decide independently that a different visit mode is clinically equivalent.

Q05 | Correct answer: D

Resolve the requested test against the signed order and obtain any needed authorized clarification.

The signed order and clarification workflow determine which test is authorized. Performing extra tests or choosing by convenience does not resolve the request discrepancy.

Q06 | Correct answer: D

Alert the provider and ensure the supplied drying and pooling precautions are completed before use.

The visible pooling means the stated preparation precautions have not been completed. Covering the wet area does not establish drying or remove the ignition hazard described by the product instructions.

Q07 | Correct answer: D

5 mm

The wheal-size field requires the raised area, measured as 5 mm in this case. The surrounding erythema is a different observation and should not replace or be added to the wheal size.

Q08 | Correct answer: B

7.2 mL

12 mg/kg x 24 kg = 288 mg; 288 mg / 40 mg/mL = 7.2 mL. The weight-based calculation must precede conversion from milligrams to milliliters.

Q09 | Correct answer: D

Remove the damaged instrument from service and obtain the suitable replacement.

The observed defect meets the supplied removal-from-service criterion. Extra cycle time or a cosmetic covering cannot make the damaged instrument meet its inspection instructions.

Q10 | Correct answer: D

25.0 kg/m2

1.8 squared = 3.24 m2, and 81 kg / 3.24 m2 = 25.0 kg/m2. Dividing by height without squaring it produces 45.0, which does not follow the supplied formula.

Q11 | Correct answer: B

Upright with the head leaning forward during the instructed compression.

The stated protocol uses an upright, forward-leaning position during soft-nose compression. Tilting backward promotes swallowing blood and does not follow the supplied procedure.

Q12 | Correct answer: B

Prepare solution within the specified temperature range and recheck it.

The prepared solution is outside the explicit procedure range. Adjusting and rechecking temperature follows the instruction; pressure changes do not correct that mismatch.

Q13 | Correct answer: A

With the hinge open and unlocked as specified.

The supplied open-position requirement allows the validated process to contact the intended surfaces. Locking or taping the instrument closed does not follow that process.

Q14 | Correct answer: C

Keep bare fingers away from the contaminated outer glove surface during removal.

Glove removal should avoid contact between bare skin and the contaminated outer surface, followed by hand hygiene. Handling the bloody exterior with bare fingers transfers the very contamination the glove was intended to contain.

Q15 | Correct answer: D

Repeat the required antisepsis and drying under the protocol.

The nonsterile contact triggers the explicitly supplied repeat-preparation rule. A glove that looks clean is not a sterile surface, and dry wiping does not repeat the required antisepsis.

Q16 | Correct answer: A

Follow the current kit's instructions and ask the clinician about medication questions.

Preparation is specific to the actual test system. Instructions from a different method should not replace the supplied kit instructions, and the assistant should not independently stop medication.

Q17 | Correct answer: B

Cleaning followed by the validated high-level disinfection process.

The supplied instructions require both cleaning and high-level disinfection for this device and use. A surface wipe or cleaning alone omits a required processing step.

Q18 | Correct answer: B

0.5 mV

5 mm / 10 mm per mV = 0.5 mV. This calculation uses the voltage scale rather than the horizontal paper-speed scale.

Q19 | Correct answer: B

Use gloves to apply a thin layer to the specified area and leave it uncovered.

The thin layer, specified location, glove use, and absence of an occlusive covering all follow the supplied directions. Adding a thick layer or occlusion changes the ordered application.

Q20 | Correct answer: D

Document the measured rate and promptly notify the clinician under the protocol.

The measurement exceeds the explicitly supplied reporting threshold even without current distress. The assistant should communicate the finding accurately without diagnosing its cause.

Q21 | Correct answer: B

Avoid forceful milking and use the permitted warming and recollection steps.

Forceful squeezing can alter the collected specimen by introducing tissue fluid and other collection effects. The supplied warming and new-lancet recollection steps are preferable to dilution or tissue trauma.

Q22 | Correct answer: A

Use the instructed deep-cough technique to collect sputum rather than saliva.

The requested specimen is lower-respiratory material obtained with the supplied deep-cough technique. Saliva, nasal secretions, and dilution with water do not meet that specification.

Q23 | Correct answer: A

Use the dedicated cuff and keep it within the designated patient area.

The dedicated cuff follows the stated contact-precaution workflow and limits equipment movement between patients. Shared or improperly relocated equipment can carry contamination outside the designated area.

Q24 | Correct answer: D

178 mm Hg

148 mm Hg + 30 mm Hg = 178 mm Hg. The target follows the supplied palpation-based procedure rather than a fixed inflation pressure unrelated to this patient.

Q25 | Correct answer: A

Wash with soap and running water.

Visible soil requires washing with soap and water. Covering it with gloves or relying on hand rub over the soil does not remove the contamination.

Q26 | Correct answer: D

Bag it with minimal handling in the designated laundry container.

Minimal handling and appropriate containment limit exposure from contaminated laundry. Shaking or moving uncontained wet linen can spread contamination to people and surfaces.

Q27 | Correct answer: A

Document the observation and obtain clinician instructions under the procedure.

Failure to meet the supplied technique criterion needs clinician review. The assistant should not invent documentation, repeat the dose independently, or create swelling by rubbing.

Q28 | Correct answer: C

Use the approved transfer device and allow transfer as directed without forceful plunger pressure.

The supplied transfer device and method limit sharps exposure and unnecessary mechanical stress on the specimen. Forcing the plunger or improvising an open transfer does not follow the validated process.

Q29 | Correct answer: A

Route the completed draft to the prescriber for authorization.

A prepared draft is not the same as a prescriber-authorized prescription. The assistant should route it through the stated approval workflow rather than sign or alter it independently.

Q30 | Correct answer: D

Use the specified observational tool and document the observed behaviors.

The supplied procedure uses structured behavioral observation when reliable self-report is unavailable. Inability to give a number does not establish absence of pain.

Q31 | Correct answer: D

Report the missed void to the laboratory and obtain instructions without labeling the collection complete.

A missed void makes the reported collection incomplete under the supplied instructions. Estimating volume or extending time independently does not restore the required timed collection.

Q32 | Correct answer: C

Document former cigarette use and current reported nicotine vaping.

Both the prior cigarette history and current reported vaping are relevant intake information. Stopping one product does not establish absence of all nicotine use.

Q33 | Correct answer: C

The tape indicates exposure to specified conditions but does not alone prove sterility of every item.

An external chemical indicator provides information about process exposure, not direct proof that all contents are sterile. It must be interpreted with the required cycle and other monitoring information.

Q34 | Correct answer: C

Use the brush and dustpan to collect fragments into the approved sharps container.

Mechanical collection avoids direct hand contact with contaminated broken glass. Gloves do not make picking up sharp fragments by hand an appropriate substitute.

Q35 | Correct answer: D

Report that the 2 mL limit is below the 3 mL requirement and obtain a revised authorized collection plan.

4 kg x 0.5 mL/kg = 2 mL total for this visit, less than the 3 mL requested. Dividing the draws does not change the total, and test priorities or volume limits need authorized review.

Q36 | Correct answer: B

Document the refusal and notify the clinician without recording the dose as given.

The record should reflect that the patient declined and did not receive the medication. The clinician should be informed; preparing a dose does not justify documenting administration or changing treatment.

Q37 | Correct answer: C

Clip as needed and prepare clean, dry skin before applying a fresh electrode.

The supplied preparation supports reliable skin contact without intentional skin injury. Lotion and stacked loose electrodes do not provide the instructed clean, dry contact surface.

Q38 | Correct answer: B

Correct the earphone placement before beginning the screening.

Matching labeled transducers to the correct ears preserves ear-specific testing and documentation. Reversed placement could assign findings to the wrong side.

Q39 | Correct answer: D

Record what occurred, stop the sequence, and obtain clinician instructions.

The event meets the supplied stop-and-contact condition, so a standard timed sequence is not established. Restarting the test or inventing a result without authorized direction is inappropriate.

Q40 | Correct answer: D

101.3 degrees F

(38.5 x 9/5) + 32 = 69.3 + 32 = 101.3 degrees F. Omitting the added 32 gives an incorrect conversion.

Q41 | Correct answer: A

Refer the requested collection to the qualified arterial collector through the clinic workflow.

The requested specimen exceeds the assistant's explicitly stated collection authorization. Routing to the qualified collector preserves the ordered specimen type without mislabeling or an unauthorized puncture.

Q42 | Correct answer: B

Stop preparation and reconcile the product identity with the order using the approved workflow.

A look-alike name does not establish that a product matches the ordered drug. The alert needs resolution before administration rather than an override or an improvised dose change.

Q43 | Correct answer: C

Reports stomach burning after meals for three days.

The entry preserves the reported symptom, trigger, and duration without adding a diagnosis. A chief complaint should not convert an unassessed symptom into a confirmed disease.

Q44 | Correct answer: C

Protect the eye from pressure and summon the clinician immediately.

An embedded ocular object requires urgent clinician management and protection from further pressure or manipulation. Attempted removal or rubbing can worsen the injury.

Q45 | Correct answer: B

Remove the expired product from use and verify an in-date replacement.

An expired product should not be selected for administration. Appearance does not establish retained quality, and increasing the dose cannot correct an expired supply.

Q46 | Correct answer: A

Set and verify zero according to the scale instructions.

An unloaded scale should meet its zero-check requirement before measurement. Correcting and verifying the device is more reliable than recording an offset measurement or copying historical data.

Q47 | Correct answer: C

Direct and assist patients through the clear marked route under the evacuation plan.

The plan specifies the nearest safe exit, so the clear marked route takes precedence over a shorter smoke-filled route. Evacuation should follow the plan without delaying for routine records or obstructing egress.

Q48 | Correct answer: C

380 L/min

The supplied protocol selects the highest technically acceptable value, 380 L/min. An average is not the requested result even though it can be calculated from the attempts.

Q49 | Correct answer: C

In the patient-reported field with the reported time and home source.

The source and time distinguish a reported home value from a measurement performed or observed by staff. Relabeling it as an office measurement misrepresents how the data were obtained.

Q50 | Correct answer: B

Replace the filled container using the approved closure and handling procedure.

The fill line is the specified replacement limit. Compressing contents, leaving sharps exposed, or emptying them into an unsuitable bag increases injury risk.

Q51 | Correct answer: C

Insert at 90 degrees into the skin fold and avoid massaging afterward.

The correct technique follows the explicit device-specific angle, skin-fold, and aftercare directions. Injection technique should not be substituted from another device or route.

Q52 | Correct answer: D

0.01 mL graduations.

A syringe with 0.01 mL graduations can measure 0.26 mL directly. The coarser scales do not mark the ordered volume without estimating or changing the dose.

Q53 | Correct answer: B

Around the widest circumference at the stated frontal and occipital landmarks.

The supplied technique measures the greatest occipitofrontal circumference using the stated landmarks. Neck, crown-to-chin, and forehead-only paths measure different dimensions.

Q54 | Correct answer: D

Put on gloves so they cover the gown cuffs.

Gloves over the gown cuffs complete the explicitly supplied donning sequence. Leaving the wrists exposed or omitting gloves does not follow the specified protection arrangement.

Q55 | Correct answer: A

Clear the cord and leave the patient safely seated with the walker and call device accessible.

Clearing the walking path and keeping assistance and mobility aids accessible reduces the identified hazard. Removing the walker or leaving the patient standing creates additional fall risk.

Q56 | Correct answer: C

Route the specimen or request to an appropriately authorized laboratory through the approved process.

The stated certificate does not authorize this moderate-complexity system. Instrument size and a clinical order do not convert the test to waived status or expand the laboratory's authorization.

Q57 | Correct answer: C

10 mL concentrate plus 90 mL water.

The specified 10 mL of concentrate and 90 mL of water total 100 mL in a 1:9 mixture. Adding 100 mL of water would produce 110 mL and a different concentration.

Q58 | Correct answer: A

Record each value with its body position and the specified measurement time.

Position and elapsed time are essential to interpreting the ordered orthostatic series. An unlabeled average removes the differences the procedure was intended to document.

Q59 | Correct answer: A

Tell the patient to call 911 now and follow the clinic's emergency-call procedure.

The supplied instructions require emergency help for these symptoms and explicitly rule out reliance on continuous real-time watching. Waiting for a monitor-team call could delay urgent evaluation.

Q60 | Correct answer: A

2,600 mg

1,950 mg + 650 mg = 2,600 mg in the specified period, which is below the stated 3,000 mg limit. The total must include all recorded acetaminophen sources rather than treating products separately.

Q61 | Correct answer: C

Notify the appropriate staff and arrange a properly collected replacement.

The visible clots violate the supplied acceptance criterion. Removing or disrupting clots does not restore a representative anticoagulated specimen for the ordered test.

Q62 | Correct answer: A

Apical pulse 102 beats/min, counted for 60 seconds.

A full 60-second count is already the number of beats per minute. The location and method should match what was actually measured; age does not justify changing the count.

Q63 | Correct answer: B

Report the discrepancy and have the provider evaluate before proceeding.

The observed count differs from the order and meets the supplied escalation rule. Blind probing can injure tissue, and documenting an unobserved removal would make the record inaccurate.

Q64 | Correct answer: A

Use comparable lower-thigh sites for both leg electrodes and document the modification.

The supplied diagram calls for comparable proximal sites with clear documentation. Disconnecting a lead or making an unrecorded asymmetric substitution does not follow that instruction.

Q65 | Correct answer: B

Use the unexpired compatible control and repeat QC as instructed before considering release.

Expired control material cannot establish valid QC for this system. Repeating QC with suitable unexpired material addresses the identified problem; patient testing remains on hold until the specified acceptance conditions are met.

Q66 | Correct answer: A

Stop and obtain an uncontaminated replacement under the medication workflow.

Contact with eyelashes can contaminate the applicator and triggers the supplied replacement policy. A casual wipe does not restore a suitable medication-delivery tip.

Q67 | Correct answer: D

Replace the contaminated basin and solution before continuing setup.

The nonsterile bottle exterior has contacted the basin interior and compromised the setup. Removing some fluid or wiping does not establish sterility of the remaining basin and solution.

Q68 | Correct answer: A

Help the patient lie on the abdomen and drape areas not needed for the examination.

Prone means lying on the abdomen and provides posterior access for the requested examination. Draping should preserve privacy while allowing the needed area to be examined.

Q69 | Correct answer: A

Reinforce the dry-dressing instruction and obtain clinician guidance about the planned shower.

The assistant should reinforce the specific current instruction and clarify how it applies to the patient's planned activity. An unapproved change in the time or wound-care method creates a new plan.

Q70 | Correct answer: B

Loosen the wrap under the procedure and promptly report the symptoms and appearance.

The new distal symptoms meet the stated procedure for immediate loosening and clinician notification. Continuing constriction or documenting normal findings could delay correction of impaired circulation.

Q71 | Correct answer: D

Replace latex-containing supplies with approved nonlatex alternatives and alert the procedural team.

Replacing latex-containing supplies and informing the team addresses the reported material allergy before exposure. Double gloving with latex or limiting the duration does not remove that material from the setup.

Q72 | Correct answer: C

Allow the site to air dry fully without touching or blowing on it.

Air drying follows the explicitly supplied preparation instructions without recontaminating the site. Blowing or wiping with clothing introduces an unapproved step.

Q73 | Correct answer: B

Resolve its traceability through the approved records and withhold release while identity is unresolved.

The supplied release procedure requires a reliable link to the actual load. Guessing an identifier creates false traceability and cannot establish that the pack met release conditions.

Q74 | Correct answer: A

Arrange provider evaluation and obtain an order before performing removal.

The supplied policy requires provider evaluation and an order before the assistant performs removal. A patient request or elapsed time does not satisfy that authorization requirement.

Q75 | Correct answer: C

Explain the need and arrange respectful access to the chest-wall landmark beneath the tissue.

Breast tissue should not replace the required chest-wall landmark. Respectful explanation, privacy, and appropriate assistance allow accurate placement without using the nipple or convenient adhesion as substitutes.

Q76 | Correct answer: A

Use the specified ice-water slurry and prompt transport without freezing.

The supplied temperature and transport method must be followed together. Freezing or using another cooling method is not an equivalent substitute for this specimen's specified handling.

Q77 | Correct answer: B

Stop the attempt, remove the needle safely, and promptly report the radiating pain.

Radiating electric-shock pain raises concern for nerve contact and calls for stopping the attempt and reporting. Continuing or probing can worsen injury and does not establish a suitable collection site.

Q78 | Correct answer: C

The hemostat.

A hemostat is designed to clamp a vessel. Tissue forceps grasp tissue but do not provide the same hinged clamping function.

Q79 | Correct answer: B

Release the tourniquet and follow the specified rest step before another attempt.

The stated continuous-application limit has been exceeded and the supplied reset procedure applies. Continuing or increasing pressure prolongs the condition the procedure is designed to avoid.

Q80 | Correct answer: B

Turn off or relocate the nonessential interference source as permitted by equipment policy and repeat the tracing.

The observed on-off relationship points to an external electrical source. Removing that source under policy addresses the artifact without changing anatomical lead placement or assigning a diagnosis.

Q81 | Correct answer: C

The right upper arm.

The right upper arm follows the explicit site instruction and avoids the fistula arm. A historical chart entry does not override the current limb-specific order.

Q82 | Correct answer: A

Keep the fist gently closed without repeated pumping.

A gently closed fist follows the supplied collection instruction without repeated muscular pumping. Vigorous repeated clenching is not needed to perform the ordered draw and can alter specimen conditions.

Q83 | Correct answer: B

Use the working manual device and address the powered-off unit under equipment policy.

A shutdown before measurement is an equipment failure, not a physiological result. An available working method permits accurate measurement while the faulty unit is handled appropriately.

Q84 | Correct answer: D

Cut close to the skin on one side and withdraw the suture without pulling the long exposed segment through tissue.

Cutting close to the skin reduces the amount of exposed material drawn through the tract. Cutting at the top of the knot can pull more externally exposed suture through the tissue.

End of Practice Exam